DIABETES IN LABOR: INSULIN AND DEXTROSE

Companion to Diabetes in pregnancy · CSM OB insulin infusion protocol (CHRISTUS Health) · ACOG PB 201 · ADA Standards 2026 §15–16 · Labor and delivery · Decision support only, not unit policy

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HOW TO READ THIS CHART: Step 1 on admission. Each FSBG: find the row, set the insulin rate AND the fluid. Hexagons are questions; shaded rectangles are actions. FSBG is read as a whole number (60 = the ≤60 row).

SHADING KEY: WHITE = monitor / set-up · LIGHT GRAY = admission, after delivery · MID GRAY = insulin running · DARK GRAY = hypoglycemia, DKA

STEP 0: DIABETES IN LABOR

GDM A1/A2, type 2 or type 1 admitted for labor, induction or scheduled cesarean. Unsure of type or insulin-deficient: treat as type 1 for basal and get senior/endocrine review.

STEP 1: ON ADMISSION (CSM) · LIGHT GRAY

Stop all insulin, including SC insulin pumps, and oral agents. NPO except 8 oz ice chips every 4 h. POC glucose; **BMP + urine ketones if FSBG >160**. **Type 1: start the IV floor BEFORE stopping SC/pump, or keep the glargine/detemir already given — never a gap.** Night-before / morning home doses and steroid adjustment: per the CSM provider/pharmacy order set (this app gives no home doses).

DRIP SET-UP · WHITE

Regular insulin (Myxredlin) **100 units/100 mL**: units/h = mL/h; range **0.5–5 units/h**. PE-lined tubing: fill with insulin, **wait 20 min, then flush**. Smart pump, dedicated line, independent double-check. High-alert.

MONITORING · WHITE

Drip running: FSBG every 1 h regardless of dilation (Michael override of CSM q2h; ACOG PB 201).
No drip: q2h latent/NPO; q1h at 4–6 cm or regular contractions (CSM).
FSBG before starting IV fluids. Hospital POC for decisions; CGM for trends only.

TYPE 1: NEVER ZERO BASAL · MID GRAY

61–90: 0.5 units/h floor + D5LR 125 + notify provider; provider may set 0.5–1 units/h by trend. **Clinician override of the CSM chart** (CSM: hold). Not needed if effective SC basal or a working pump is on board; do not stack. Stop only for ≤60 or symptoms = **emergency pause**; resume 0.5 once >70 and rising; pause >~1 h needs provider review.

STEP 2: FSBG NOW — WHICH ROW? (CSM NOMOGRAM, MAINTENANCE 125 mL/h)

Look up the current FSBG every check; move rows as glucose changes. No bolus. Max 5 units/h. Never use this chart to treat DKA.

FSBG (mg/dL)	GDM (A1/A2) and type 2: insulin	Type 1: insulin	Fluid at 125 mL/h
≤60 (CSM "<60" row; 60 included)	**No insulin. Contact provider. Hypoglycemia protocol (below).**	**Emergency pause** (dextrose continues); resume 0.5 once >70 and rising.	**D5LR**
61–90 (target)	Hold infusion (A1: no insulin; monitor).	**0.5 units/h floor** (provider 0.5–1) + notify · *override of CSM*	**D5LR**
91–120	**0.5 units/h**	**0.5 units/h**	**D5LR**
121–150	**1 units/h**	**1 units/h** · watch for ketones on LR	**LR**
151–180	**2 units/h**	**2 units/h** · watch for ketones on LR	**LR**
181–220	**3 units/h**	**3 units/h** · watch for ketones on LR	**LR**
221–250	**4 units/h**	**4 units/h** · watch for ketones on LR	**LR**
>250	**5 units/h** · max: notify provider	**5 units/h** · watch for ketones on LR	**LR**

HYPOGLYCEMIA <70 OR SYMPTOMS: RESCUE NOW (CSM ADULT HYPOGLYCEMIC PROTOCOL) · DARK GRAY

D50 50 mL IV, as needed: FSBG <50 in an alert patient unable to swallow; OR questionable level of consciousness with FSBG <50 (CSM pharmacy).
D50 25 mL IV push, as needed: FSBG 50–69 in an alert patient who is NPO or unable to swallow; OR questionable level of consciousness with FSBG 50–69 (CSM pharmacy).
Glucagon (GlucaGen) 1 mg IM, ONCE (do not repeat): FSBG <70 in an alert patient with no IV access or unable to swallow; OR FSBG <60 with a questionable level of consciousness and no IV access.
Glutose 40% oral gel 15 g buccal: FSBG 50–59 in an ALERT patient without IV access.
Glutose 40% oral gel 30 g buccal: FSBG <50 in an ALERT patient without IV access.
Recheck in **15 min**; repeat until >70. GDM/type 2: hold IV insulin. Don't delay symptomatic rescue for a lab value.

SICK, KETONES, VOMITING? → DKA · DARK GRAY

Pregnancy DKA can occur below 200. Check ketones (BHB), VBG, electrolytes/K+, anion gap; fetal monitoring. **Use the CSM DKA order set, not this chart.** Reference only, per CSM DKA order set: insulin 0.1 units/kg/h after K+ is safe. Type 1 on LR with ketones: here, no ad hoc dextrose.

A · AFTER DELIVERY (CSM)	Notify the provider when delivery is complete to start the **CSM Postpartum Diabetes Management Order Set** (doses there). GDM: stop insulin at placenta; OGTT 75 g at 4–12 weeks. Type 1: basal always covered. Continue IV insulin **1–2 h after the first SC basal dose**; then stop D5LR and insulin together.
B · NEWBORN	Tell the nursery: diabetes type, treatment, last maternal glucose, steroids. Stable baby: feed within 1 hour; neonatal glucose protocol belongs to the nursery team.
C · STEROIDS / TIMING	Don't withhold indicated steroids for hyperglycemia; SMFM #58: no late-preterm steroids in pregestational diabetes. Delivery timing and EFW ≥4500 g counseling stay on the Diabetes in pregnancy card.
D · NOTIFY	RN to OBHG hospitalist: FSBG ≤60, 61–90 on a type 1 floor, 5 units/h with rising values, ketones, or a gap in type 1 basal.

Doses to be checked against CSM pharmacy build. Decision support only, not orders or unit policy. FSBG = fingerstick blood glucose; BHB = beta-hydroxybutyrate. **Cite:** CHRISTUS Health OB insulin infusion protocol (CSM, transcribed 11 Nov 2026); ACOG PB 190 and 201; ADA Standards of Care 2026 §15–16; SMFM Consult #58.

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